

PHYSIOTHERAPY CLINICAL CASE STUDY REPORT

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CASE INFORMATION

Case Number:	Auto-assigned
Category:	Outdoor
Date:	2024-07-30
Referring Physician:	Self-referred / Not specified

PATIENT DEMOGRAPHICS

Name:	Rukhsana Bibi
Age:	55 years
Gender:	Female
Marital Status:	Married
Language:	Not Provided
Occupation:	Housewife
Address:	Rawalpindi
Mode of Admission:	Outdoor / OPD

PRESENT COMPLAINT

The patient, a 55-year-old female, presents with a chief complaint of right-sided neck pain and associated stiffness, which has been ongoing for approximately one month. The pain is reported to be moderate in intensity at rest and significantly worsens with specific neck movements.

HISTORY OF PRESENTING COMPLAINT

The patient reports a gradual onset of right-sided neck pain and stiffness approximately one month prior to presentation. She denies any history of trauma, sudden jerk, or specific injury preceding the onset of symptoms. The pain is described as aggravating with neck rotation and looking upwards, reaching an intensity of 6 out of 10 on the Numeric Pain Rating Scale (NPRS). She finds relief with rest and the application of a hot pack to the affected area. The pain and stiffness interfere with her daily household activities, particularly those requiring sustained neck postures or repetitive movements. There are no reported previous episodes of similar symptoms.

PAIN PROFILE

Location:	Right lower cervical region, specifically over the paraspinal muscles.
Onset:	Gradual
Duration:	1 month
Nature:	Dull aching pain with associated stiffness, occasionally sharp with movement.
Radiation:	No radiation to upper limbs
NPRS Score:	6/10 at rest, 8/10 on movement

Aggravating Factors

PHYSIOTHERAPY CLINICAL CASE STUDY REPORT

CaseGen AI | Generated: 16 May 2026 20:31

- Neck rotation
- Looking up

Relieving Factors

- Rest
- Hot pack application

PAST HISTORY

Medical History:	Mild hypertension, managed conservatively.
Surgical History:	No previous surgeries reported
Medications:	Not Provided
Family History:	Not Provided
Socioeconomic:	Not Provided
Pre-morbid Status:	Independent in all Activities of Daily Living (ADLs) prior to current episode, actively managing household responsibilities.
Growth & Development:	Not applicable / within normal limits

PATIENT'S PRIORITIES & FUNCTIONAL GOALS

- Reduce neck pain to comfortably perform daily household chores.
- Improve neck mobility to look over her shoulder without pain, especially for tasks like driving or cooking.
- Achieve comfortable sleep without being disturbed by neck pain and stiffness.
- Return to full independence in all household activities without pain or limitation.

GENERAL HEALTH STATUS

Vitals

Blood Pressure:	Normal / Within limits
Pulse:	Normal / Within limits
Respiratory Rate:	Normal / Within limits
Temperature:	Afebrile
O2 Saturation:	Normal / Within limits
Weight:	Not Provided
Height:	Not Provided
BMI:	Not Provided

Clinical Examination

Lymph Nodes:	Not palpable
Jaundice:	Absent
Pallor:	Absent
Cyanosis:	Absent
Edema:	Absent

PHYSIOTHERAPY CLINICAL CASE STUDY REPORT

CaseGen AI | Generated: 16 May 2026 20:31

Clubbing:

PHYSIOTHERAPY CLINICAL CASE STUDY REPORT

CaseGen AI | Generated: 16 May 2026 20:31

Absent

Overall Impression:

The patient is conscious, alert, and hemodynamically stable, presenting in no acute distress.

GENERAL HEALTH CONDITION

Malaise: None reported

Fatigue: None reported

Fever: Afebrile

Weight Changes: None reported

Sleep: Sleep is disturbed due to neck pain and stiffness.

Cognition: Intact

Mood / Affect: Cooperative and oriented

SYSTEMS REVIEW

Cardiovascular: No complaints reported

Pulmonary: No complaints reported

Gastrointestinal: No complaints reported

Urinary: No complaints reported

Integumentary: Skin intact, no wounds or lesions noted in the cervical region.

Endocrine: No complaints reported

Reproductive: Not assessed / Not applicable

MUSCULOSKELETAL SYSTEM

Gross Symmetry: Normal, no obvious gross asymmetry noted.

Posture: Guarded posture of the cervical spine noted, specific details regarding alignment not explicitly documented but likely consistent with degenerative changes.

Deformity: None observed

Swelling: None observed

Tenderness: Grade 2 tenderness over the right lower cervical paraspinal muscles on palpation.

Muscle Wasting: None observed

Muscle Spasm: Palpable spasm noted in the right lower cervical paraspinal muscles.

Cervical ROM: Extension and right rotation are restricted and painful. Flexion, left rotation, and lateral flexion are within functional limits but may be guarded.

Lumbar ROM: Normal

Upper Limb ROM: Normal

Lower Limb ROM: Normal

Muscle Strength: Normal throughout upper and lower limbs, no specific weakness noted.

Special Tests

- Not Provided: Not performed or documented

PHYSIOTHERAPY CLINICAL CASE STUDY REPORT

CaseGen AI | Generated: 16 May 2026 20:31

NEUROLOGICAL ASSESSMENT

Consciousness:	Conscious and oriented to time, place and person
Numbness / Tingling:	None reported
Reflexes:	Normal deep tendon reflexes in upper and lower limbs.
Muscle Tone:	Normal
Coordination:	Normal
Gait:	Normal
Cranial Nerves:	Intact, not specifically assessed but inferred from overall neurological status.
Upper Motor Neuron:	No signs of Upper Motor Neuron (UMN) lesion.
Lower Motor Neuron:	No signs of Lower Motor Neuron (LMN) lesion, specifically no radicular symptoms or motor weakness in upper limbs.

FUNCTIONAL STATUS

ADL Status:	Partially dependent, experiencing limitations in ADLs requiring neck movement or sustained postures.
Mobility:	Independent, but with guarded cervical movements.
Work Status:	Modified duties, experiencing limitations in household chores due to neck pain.
Recreational:	Limitations in recreational activities if any, not explicitly reported but inferred from pain and stiffness.
Assistive Devices:	None currently in use

Functional Goals

- Perform overhead tasks and reach for objects without pain.
- Drive comfortably and safely by being able to check blind spots.
- Engage in social activities and hobbies without neck discomfort.

INVESTIGATIONS & IMAGING

Imaging:	None provided / Not available
Lab Tests:	None provided / Not available
EMG / NCS:	Not assessed
Other:	None

CLINICAL IMPRESSION & DIAGNOSIS

Cervical Spondylosis with associated muscle spasm (ICD-10: M47.812 - Other spondylosis with radiculopathy, cervical region, or M47.91 - Spondylosis, unspecified, cervical region, with M62.830 - Muscle spasm of neck).

HYPOTHESIS / PROBLEM STATEMENT

The patient presents with chronic right-sided cervical pain and stiffness, indicative of cervical spondylosis, likely exacerbated by muscle spasm in the paraspinal region. This condition is causing significant functional limitations in daily activities requiring neck movement, such as household chores and potentially sleep. The absence of neurological deficits suggests a localized

PHYSIOTHERAPY CLINICAL CASE STUDY REPORT

CaseGen AI | Generated: 16 May 2026 20:31

musculoskeletal issue rather than nerve root compression. The treatment approach will focus on pain reduction, improving cervical range of motion, reducing muscle spasm, and enhancing functional independence through a combination of modalities, manual therapy, and therapeutic exercises, alongside patient education on posture and ergonomics.

TREATMENT PLAN

Short-Term Goals (0 - 2 Weeks)

- Reduce right cervical pain to 3/10 or less on NPRS at rest within 2 weeks.
- Increase cervical extension and right rotation range of motion by 25% within 2 weeks.
- Decrease palpable muscle spasm in the right lower cervical paraspinal muscles within 2 weeks.

Long-Term Goals (3 - 6 Weeks)

- Achieve full pain-free cervical range of motion in all planes within 6 weeks.
- Enable the patient to perform all household activities without pain or limitation within 6 weeks.
- Educate the patient on self-management strategies to prevent recurrence and maintain cervical health long-term.

Physiotherapy Interventions

- Moist Heat Therapy: 15 minutes to the right cervical region to promote muscle relaxation and pain relief.
- Gentle Cervical Manual Mobilizations: Grade I-II oscillations for pain modulation, joint nutrition, and to improve articular mobility in restricted segments.
- Soft Tissue Release/Massage: To the right cervical paraspinal muscles to reduce spasm and tenderness.
- Cervical Isometric Exercises: Submaximal contractions for flexion, extension, and rotation, 3 sets of 10 repetitions, hold 5 seconds, performed within pain-free limits to improve muscle endurance and stability.

Home Exercise Program

- Chin tucks: 3 sets x 10 repetitions, hold 5 seconds, to strengthen deep neck flexors and improve posture.
- Scapular retractions: 3 sets x 10 repetitions, hold 5 seconds, to improve thoracic posture and scapular stability.
- Gentle active range of motion exercises for the cervical spine (flexion, extension, lateral flexion, rotation) within pain-free limits, 10 repetitions each direction, 3 times daily.
- Self-application of heat pack to the neck for 15-20 minutes as needed for pain relief.

Patient Education

- Ergonomic workstation and household task setup advice to minimize cervical strain and promote optimal posture.
- Instruction on proper sleeping posture and pillow selection to support the cervical spine.
- Pain management strategies including the appropriate use of heat therapy and activity modification.
- Importance of regular, gentle movement and avoiding prolonged static postures.

Referrals:	None at this time
Follow up:	Review in 1 week to assess progress and adjust treatment plan. Full reassessment at 3 weeks.
Prognosis:	Good prognosis for significant improvement in pain and function with consistent adherence to the physiotherapy program, given the absence of neurological compromise and positive response to relieving factors.

Physiotherapist Signature

Date: 16 May 2026

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PHYSIOTHERAPY CLINICAL CASE STUDY REPORT

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